



SRI LAKSHMI NARAYANA INSTITUTE OF MEDICAL SCIENCES

OSUDU, AGARAM VILLAGE, VILLIANUR COMMUNE, KUDAPAKKAM POST, PUDUCHERRY - 605 502.

DEPARTMENT OF GENERAL MEDICINE DISCHARGE SUMMARY AT REQUEST

NAME	MR. [REDACTED]	UHID	[REDACTED]
AGE/SEX	[REDACTED]	IP NO	[REDACTED]
ADDRESS	2, MARIYAMMAN KOVIL ST,BEEMARAO NAGAR , VILLIANUR		

D.O.A : 06/12/2024

D.O.D : 09/12/24

DIAGNOSIS:

1. CHRONIC LIVER DISEASE WITH ASCITES
2. BILATERAL PNEUMONIA
3. OLD HEMORRHAGIC CVA
4. S HTN

CHIEF COMPLAINTS:

C/O fever × 2days,
C/O loose stools × 1day ,
C/o altered sensorium × 1day,

HISTORY OF PRESENTING ILLNESS:

Patient was apparently normal 2days back after which he developed
C/O fever ,intermittent in nature associated with chills and rigor relieved on medication × 2days,
C/O loose stools-3 episodes ,semi solid ,not blood stained,
C/o shortness of breath+
C/o dry cough+
NO H/O Vomiting ,chest pain,palpitation,
NO H/O burning micturition ,

PAST HISTORY:

K/c/o chronic alcoholic × 30 years- Last binge before 3 month back ,
K/c/o chronic smoker×30 years,
P/H/O admission @slims 22/08/24 to 31/08/24-
Old hemorrhagic CVA -2° hepatic coagulopathy ,
Chronic liver disease
ADS/HTN,

K/C/O HYPERTENSION × 10 years on T.AMLONG,
K/C/O CLD (ethanol related).

PERSONAL HISTORY:

Mixed diet,
Normal sleep pattern,
Normal bladder habits,

ON EXAMINATION:**General Examination –**

Patient is conscious, not oriented /confused
icterus+,
Pedal edema+,
No pallor , clubbing,cyanosis, lymphadenopathy,
B/L LL Pigmentation present,

VITALS:

PR -74 beats/min
BP- 140/70 mmhg
SpO2-96%@4LO2

S/E:

CVS: S1S2(+), no murmur

RS : B/L basal crepts+

P/A : Soft, distended ,fluid thrill+ , tenderness@ RIF & Epigastric ,

CNS :E4 V1 M6 ,B/L ERTL, Confused ,fine minimal tremors +



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INVESTIGATIONS:

CBC	06/12/24
RBC	4.31 Million/c
HB	14.6 gms%
PLT	213 10×3 /mcgL
WBC	16.41
NEU	72.6%
LYM	11.0%
MON	11.9%
EOS	4.1%
BAS	0.4%

RFT	06/12/2024	07/12/24
Urea	27.7 mg/dl	19.9mg/dl
Creatinine	0.90 mg/dl	0.78mg/dl

S.ELECTROLYTES	06/12/2024	07/12/2024
Sodium	132mmol/L	136mmol/L
Potassium	4.2mmol/L	4.1mmol/L
Chloride	98 mmol/L	101mmol/L

PT-INR(07/12/24):

TEST- 22.0

CONTROL-11.6

INR-1.96

LFT	06/12/2024
ALT	22.7U/L
AST	44.8U/L
ALP	74U/L
BILIRUBIN DIRECT	1.50mg/dl
BILIRUBIN TOTAL	3.90mg/dl
ALBUMIN	2.39g/dl
TOTAL PROTEIN	7.09g/dl

USG ABDOMEN (23/10/24)

Liver : normal ,13.4 cm

GB : normal

Pancreas : Normal

Kidney : normal

Spleen : Appears prominent,12.3CM in size

Bladder : minimally distended

Prostate : appears normal in size

Rt iliac fosa:Appendix Is not visualised , no evidence of mass formation / abscess . Moderate left pleural effusion

Impression:

- Prominent spleen
- Moderate left pleural effusion

HRCT -THORAX :

- Mild to moderate left pleural effusion.
- Subsegmental collapse and atelectatic strands seen in basal segments of left lung



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upper lobe and most of the lower lobe.

- Multiple small mediastinal nodes noted with some of them with calcifications, largest measuring ~ 19 x 15 mm in the pretracheal region.
- Mild cardiomegaly. Main pulmonary artery measures 27mm, is not dilated.
- Atheromatous wall calcifications seen in aortic arch and its branches.

ECHO REPORT :

- LA dilated
- (N) LVSF | EF- 65% | Grade 1 LVDD
- No RWMA
- Heavily calcified mitral leaflet
- Posterior mitral annular calcification
- Mild AS (25/13mmHg)
- Trivial MR | Trivial TR
- PASP- 16mmHg
- IVC- Normally collapsing

CONDITION AT THE TIME OF DISCHARGE:

VITALS:

PR - 80 beats / min

BP- 110/70 mmHg

SpO2 -96% at RA

S/E:

CVS : S1S2(+), No murmur

RS : Bilateral basal crepts+

P/A : Soft, distended

CNS : E4 V5 M6

COURSE IN HOSPITAL:

Patient came with above mentioned complaints and routine investigations were done and treated with oxygen support for 2 days and gradually tapered and stopped. IV antibiotics Piptaz, inj ciprofloxacin given for 4 days. Nebulizations I/v/o sever PNEUMONIA given. ECG showed T wave inversion in V2, V3, cardiology opinion obtained and advised to continue the same treatment. Further supportive treatment given and respiratory medicine opinion obtained and advised for USG guided pleural aspiration for diagnostic purpose but patient and patient attender requesting for discharge hence being DISCHARGED AT REQUEST with the following advice.



ADVICE ON DISCHARGE:

S.NO.	Name of the drug	Dosage	M	A	N	B/F	A/F	No. Of days
1.	T.COPROFLOXACIN	500mg	1	0	1		✓	3 days
2.	T.AZITHRO	500mg	1	0	0		✓	3 days
3.	T.PANTOP	40mg	1	0	0	✓		3 days
4.	T.MONTEK AB		0	0	1		✓	5 days
5.	T.ALDACTONE	50mg	1	0	0		✓	To continue
6	Syp.LACTULOSE		-	-	10ml		✓	3 days
7.	T.UDILIV	300mg	1	0	1		✓	To continue
8.	High Resource Protein powder in water		1	1	1		✓	1 month

•REVIEW in Medicine OPD SOS/ after 1 week.

IN CASE OF EMERGENCY CONTACT THE FOLLOWING NUMBERS

EMERGENCY NO- 0413- 2299200 (8:00 AM TO 4:00 PM)

